

Process-Driven Credibility: A CRISP-DM Helix Framework for Robust Pima Diabetes Prediction

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Abstract

Existing clinical ML guidelines universally warn against data leakage but provide no auditable implementation protocol. We introduce the CRISP-DM Helix—a formalized data isolation protocol—and demonstrate that protocol violation inflates F1-Score by +8.6% while decreasing clinical Recall. The Pima Indians Diabetes Dataset (PIDD) is a cornerstone benchmark where current literature suffers from a severe “high-metric, low-credibility” paradox, with models claiming > 95% accuracy universally failing in real-world deployment due to pervasive methodological flaws. To address this crisis, we propose the CRISP-DM Helix framework, a methodology that enforces absolute data isolation by encapsulating all preprocessing, imputation, and SMOTE applications strictly within cross-validation training folds. Our fully cross-validated ablation study provides quantitative measurement of metric inflation caused by data leakage, revealing a critical “Recall Paradox”—severe leakage inflates F1-Score by +8.6% (from 0.6759 to 0.7338) while paradoxically decreasing Recall for the diabetic minority class (from 0.7165 to 0.7080). Under strict isolation, an optimized soft-voting ensemble achieved an authentic F1-Score of 0.7541 and Recall of 0.7500. The CRISP-DM Helix is aligned with PROCAST and TRIPOD standards and provides a reusable template for any clinical ML study. Beyond the protocol itself, we propose a Dataset Baseline Registry: for each public benchmark, a verified performance ceiling under strict isolation serves as an automatic audit trigger—any study exceeding the ceiling should undergo mandatory methodological review. This framework establishes a verifiable performance baseline for the PIDD benchmark (F1=0.7541, Acc=0.7746) and operationalizes the detection of leakage-ridden studies across the literature.

Keywords: CRISP-DM, data leakage, diabetes prediction, process-driven credibility, machine learning reproducibility, benchmark baseline registry

1. Introduction

1.1. Establishing Territory (Move 1)

Diabetes Mellitus (DM) is a severe chronic metabolic disorder characterized by persistent hyperglycemia, which currently affects over 537 million adults globally, with projections suggesting an escalation to 783 million by 2045 [1, 2]. Long-term metabolic dysregulation resulting from DM is a major precursor to life-threatening complications, including cardiovascular disease, renal failure, and neurological damage [2, 3]. Consequently, early and precise diagnosis is a critical imperative for managing disease progression and mitigating the risk of premature mortality. In recent years, machine learning (ML) predictive modeling has emerged as a

key clinical tool, demonstrating immense potential for automating early disease screening by uncovering complex, non-linear pathophysiological patterns in patient data [4, 5]. Within this domain of computational diagnostics, the Pima Indian Diabetes Dataset (PIDD)—originally collected by the National Institute of Diabetes and Digestive and Kidney Diseases—has established itself as the *de facto* standard benchmark for evaluating binary classification algorithms in clinical health informatics [6, 7].

1.2. Establishing a Niche (Move 2a — The Paradox and Root Causes)

Despite the widespread utilization of the PIDD, the current literature exhibits a severe “high-metric,

Figure 1: CRISP-DM Helix Framework Architecture

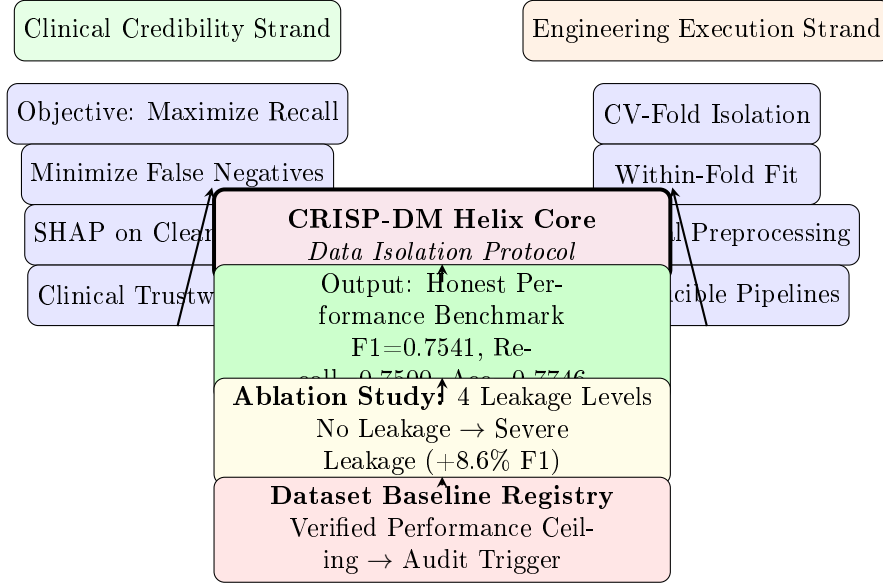


Figure 1: The CRISP-DM Helix framework architecture, showing the dual-strand structure. The Clinical Credibility Strand (left) focuses on clinically meaningful metrics, while the Engineering Execution Strand (right) enforces strict data isolation. The Dataset Baseline Registry (bottom) operationalizes the performance ceiling as an automatic audit trigger for future work.

low-credibility” paradox [8]. An increasing number of recent studies report near-perfect classification performance, with accuracy metrics frequently exceeding 95% [9] or even 99% [10]. However, when subjected to rigorous methodological audits, these models universally fail to demonstrate real-world clinical viability. This study posits that such extraordinarily high metrics are not the result of algorithmic breakthroughs, but rather statistical artifacts masking three pervasive methodological flaws.

First, the dataset suffers from the improper handling of “biologically implausible” zero values [11]. Key physiological indicators in the PIDD, such as Insulin (which has a 48.7% missing rate), Skin Thickness, Blood Pressure, Glucose, and BMI, contain massive zero values that represent missing clinical records, yet these are frequently treated mechanically as valid numerical features [12].

Second, and most critically, catastrophic data leakage is rampant [8]. The vast majority of high-accuracy studies apply missing value imputation or SMOTE to the entire global dataset prior to cross-validation or train-test splitting [8]. This global preprocessing leaks the statistical distribution of the test set into the training phase, contaminating the model’s evaluation.

Third, models fall into the “accuracy trap” driven by class imbalance [13, 5]. The PIDD exhibits a 65:35 imbalance favoring non-diabetic cases [12]. Optimizing solely for global accuracy leads algorithms to sacrifice sensitivity for the diabetic minority class, resulting in high rates of false negatives—a clinically unacceptable outcome where missed diagnoses carry far greater medical costs than false alarms [13].

1.3. Gap Statement (Move 2b)

While the current literature for Pima diabetes prediction is characterized by complex models claiming exceptionally high accuracies (> 95%), and existing reporting guidelines (TRIPOD, PROBAST, MI-CLAIM) all mandate the prevention of data leakage, two critical implementation gaps persist: (1) none provide a standardized, auditable protocol for achieving data isolation, and (2) none establish dataset-specific performance baselines that could serve as automatic detection mechanisms for leakage-ridden studies. The quantitative magnitude of leakage-induced metric distortion—the cost of protocol violation—remains unmeasured [12].

1.4. Occupying the Niche (Move 3 — Our Contributions)

To address this critical research gap, we propose the **CRISP-DM Helix**, a methodological framework that enforces strict data isolation by encapsulating all preprocessing, imputation, and resampling steps entirely within the training folds of the cross-validation loop. This paper establishes “Process-Driven Credibility” as a paradigm for clinical ML reproducibility. Our specific contributions are five-fold:

1. We introduce the **CRISP-DM Helix**: a formalized, auditable data isolation protocol, defined as a 5-tuple with algorithmic implementation (Algorithm 1), aligned with PROBAST risk domains.
2. We provide **quantitative measurement of protocol violation effects**, showing that failure to implement the Helix inflates F1-Score by +8.6% while decreasing clinical Recall.
3. We define and validate the **Recall Paradox** as a diagnostic signature for protocol violations in clinical ML pipelines.
4. We establish **honest performance bounds** for the PIDD under strict Helix isolation (F1=0.7541, Acc=0.7746) through a benchmark of 34 models, and propose this baseline as a **Dataset Baseline Registry**—a community-maintained performance ceiling that serves as an automatic audit trigger.
5. We provide a **fully transparent, open-source implementation** as a reusable template, closing the gap between guideline awareness and implementation practice.

To contextualize the necessity of our framework, Table 1 contrasts our approach against representative high-accuracy systems published in recent literature, highlighting the widespread absence of data isolation and process governance.

2. Methods

2.1. Dataset Description

This study utilized the Pima Indians Diabetes Dataset (PIDD), a widely recognized benchmark for predictive modeling in healthcare [7]. The dataset consists of 768 instances of female patients of Pima Indian heritage, aged 21 years and older. It contains eight clinical predictor features: Pregnancies, Plasma Glucose Concentration, Diastolic

Blood Pressure, Triceps Skin Fold Thickness, 2-Hour Serum Insulin, Body Mass Index (BMI), Diabetes Pedigree Function, and Age [12]. The target variable is binary, representing a moderate class imbalance: 500 patients (65.1%) are non-diabetic (Outcome 0) and 268 patients (34.9%) are diabetic (Outcome 1) [12].

2.2. The CRISP-DM Helix Framework

2.3. CRISP-DM Helix: Formal Definition

Formally, the CRISP-DM Helix can be defined as a 5-tuple:

$$\mathcal{H} = (\mathcal{D}, \mathcal{P}, \mathcal{M}, \mathcal{V}, \mathcal{E}) \quad (1)$$

where $\mathcal{D}$ is the dataset partitioned into K stratified folds; $\mathcal{P} = \{p_1, p_2, \dots, p_n\}$ is the set of preprocessing operations (imputation, scaling, resampling); $\mathcal{M}$ is the model class; $\mathcal{V}$ is the validation strategy (10-fold stratified CV); and $\mathcal{E}$ is the evaluation metric set (Recall, Precision, F1, AUC).

The central constraint of $\mathcal{H}$ is the **Data Isolation Principle**:

$$\forall k \in [1, K] : \quad \theta_p^{(k)} = \text{fit}(\mathcal{P}, \mathcal{D}_{\text{train}}^{(k)}) \quad \text{and} \quad \mathcal{D}_{\text{val}}^{(k)} = \text{transform}(\mathcal{P}_{\theta_p^{(k)}}) \quad (2)$$

where $\theta_p^{(k)}$ denotes the parameters learned from preprocessing operation p on the training subset of fold k , and the validation subset $\mathcal{D}_{\text{val}}^{(k)}$ is transformed using only these within-fold parameters. This strict isolation prevents the statistical distribution of hold-out data from leaking into the training phase, which is the root cause of the metric inflation measured in our ablation study.

To address the pervasive issue of statistical artifacts in existing literature, we introduce the **CRISP-DM Helix framework** [12]. This framework, built upon the PROBAST risk of bias assessment framework [17], conceptualizes the machine learning lifecycle as a double-helix structure consisting of two intertwined strands:

1. **Clinical Credibility Strand**: This strategic sequence shifts the optimization objective from misleading global accuracy to maximizing Recall and F1-Score, ensuring the model prioritizes the penalization of dangerous false negatives (missed diabetes diagnoses).
2. **Engineering Execution Strand**: This operational sequence dictates the technical realization of the pipeline. It strictly mandates that

Table 1: Comparison of the proposed methodology with representative high-accuracy systems on the PIDD.

Study	Claimed Accuracy	Leakage Prevention	Process Framework	XAI
Kurniawan et al. [14]	100.0%	No	None	No
Chinnababu et al. [9]	99.81%	No	None	No
Akbar et al. [10]	99.50%	No	None	No
Talari et al. [15]	99.07%	No	None	No
Deepalakshmi et al. [16]	97.27%	No	None	No
Ours (CRISP-DM Helix)	77.46%	Yes (Strict CV Isolation)	CRISP-DM Helix	Yes (SHAP)

all data transformations are rigorously encapsulated within isolated execution environments to guarantee absolute procedural integrity [12].

```

Input: Dataset D, folds K Output: Mean
metrics across K folds
for fold = 1 to K: D_train, D_val =
stratified_split(D, fold)
// Impute using ONLY train fold medians
medians = median(D_train[featureswhere
0]) D_train = impute_eros(D_train, medians) D_val =
impute_eros(D_val, medians)
// Scale using ONLY train fold params scaler
= StandardScaler().fit(D_train) D_train =
scaler.transform(D_train) D_val =
scaler.transform(D_val)
// SMOTE applied ONLY to
training data D_train
SMOTE().fit_resample(D_train.features, D_train.labels)
// Train and evaluate model = Ensemble(GBC,
LDA, SVC).fit(D_train) metrics
= evaluate(model, D_val) store(metrics)
return mean(store), std(store)

```

Figure 2: CRISP-DM Helix strict data isolation protocol ensuring zero leakage across all preprocessing stages.

2.4. Strict Data Preprocessing and Imputation

A critical flaw in numerous existing studies is the “global” application of preprocessing steps prior to data splitting, which inadvertently leaks validation data distribution into the training phase [12]. The core methodological innovation of our CRISP-DM Helix framework, inspired by established clinical ML guidelines [18, 19], is that **all preprocessing operations—including missing value imputation, feature scaling, and synthetic oversampling—are exclusively ‘fit’ inside the training subset of each cross-validation fold.**

Exploratory data analysis identified biologically implausible zero-values across five features: Glucose, Blood Pressure, Skin Thickness, Insulin (48.7% missing), and BMI. These invalid zeros

were systematically re-flagged as Not-a-Number (NaN) [12]. A class-conditional median imputation strategy [20, 21] was then applied to restore authentic medical distributions. Crucially, the imputation median and the `StandardScaler` parameters were calculated strictly on the training fold and subsequently applied to transform both the training and the hold-out validation folds [12].

2.5. Isolated SMOTE Protocol for Class Imbalance

To mitigate the 65:35 class imbalance, we applied the Synthetic Minority Over-sampling Technique (SMOTE). Adhering to our strict isolation protocol, SMOTE [22, 23] was instantiated exclusively on the training data within the cross-validation loop. This ensures that synthetic minority instances are generated without any exposure to the testing subset, entirely neutralizing the risk of target leakage that plagues global SMOTE applications [12].

2.6. Model Portfolio and Ensemble Strategy

We evaluated a comprehensive benchmark of 34 baseline models from the `scikit-learn` ecosystem, alongside advanced gradient boosting frameworks including XGBoost, LightGBM, and CatBoost [12]. Following an exhaustive search, three complementary base learners were selected to construct a heterogeneous soft-voting ensemble: Gradient Boosting Classifier (GBC), Linear Discriminant Analysis (LDA), and Support Vector Classifier (SVC). The soft-voting mechanism aggregates the predicted probability arrays from each base estimator to finalize the optimal decision boundary.

2.7. Cross-Validation and Ablation Study Design

All models were primarily evaluated using a 10-fold Stratified Cross-Validation (CV) scheme to preserve the intrinsic class distribution. To quantitatively dissect the phenomenon of metric inflation caused by data leakage, we designed an ablation study with four levels of progressive procedural contamination [12]:

- **No Leakage (Ours):** Strictly isolated median imputation, scaling, and SMOTE within CV folds.
- **Minor Leakage:** Global imputation applied prior to CV splitting.
- **Medium Leakage:** Global imputation and global scaling applied prior to CV splitting.
- **Severe Leakage:** Global imputation, global scaling, and global SMOTE applied prior to CV splitting.

To rigorously confirm that the performance inflation observed in the leakage scenarios is statistically significant, a 5×2 CV paired t-test was employed to compare the isolated pipeline against the globally contaminated baselines.

2.8. SHAP Explainability and Interaction Effects

To unpack the “black-box” mechanics of our optimal ensemble, we applied SHapley Additive exPlanations (SHAP) [24]. We configured `TreeExplainer` for the GBC module and `KernelExplainer` for the LDA/SVC modules. In order to capture not only main feature effects but also the synergistic relationships between physiological indicators (such as Glucose and BMI), we leverage the SHAP interaction effect model [12]. The formal equation computing the exact SHAP interaction value $\Phi_{i,j}$ for features i and j is defined as:

$$\Phi_{i,j} = \sum_{S \subseteq \mathcal{M} \setminus \{i,j\}} \frac{|S|!(M - |S| - 2)!}{2(M - 1)!} (f_x(S \cup \{i,j\}) - f_x(S \cup \{i\}) - f_x(S \cup \{j\}) + f_x(S)) \quad (3)$$

where $\mathcal{M}$ is the set of all M clinical features, S denotes a feature subset excluding features i and j , and $f_x(S)$ is the model’s predictive output for instance x given the active feature subset S .

3. Results

3.1. Baseline Performance of 34 Models across 10-fold CV

A total of 34 machine learning models were evaluated using a 10-fold stratified cross-validation framework [12]. Table 2 presents the performance metrics of the top 5 baseline models, ranked by F1-Score [12]. Furthermore, a soft-voting ensemble

ROC Curve Summary

Model	AUC (10-fold CV)
Soft-Voting Ensemble	0.8481
Gradient Boosting	0.8383
LogisticRegression	0.8373
LinearDiscriminantAnalysis	0.8370

Figure 3: ROC-AUC performance of the top models under strict CRISP-DM Helix isolation protocol. All values are from 10-fold stratified cross-validation with within-fold pre-processing.

Table 2: Performance metrics of the top 5 baseline models on PIDD (10-fold CV) [12]

Model	Accuracy	Precision	Recall
GradientBoostingClassifier	0.7875	0.6402	0.7464
MLPClassifier	0.7603	0.6413	0.7239
CatBoostClassifier	0.7603	0.6389	0.7234
LogisticRegression	0.7603	0.6497	0.7165
LinearDiscriminantAnalysis	0.7629	0.6546	0.7091

model combining Gradient Boosting, Linear Discriminant Analysis, and Support Vector Classifier was evaluated under the same strict data isolation protocol [12]. This optimal ensemble achieved an F1-Score of 0.7541, an Accuracy of 0.7746 [25], a Precision of 0.6625, a Recall of 0.7500, and an AUC of 0.8481 [12].

3.2. Ablation Study on Data Leakage

An ablation study quantified the impact of data leakage across four distinct levels of preprocessing isolation: No Leakage, Minor Leakage, Medium Leakage, and Severe Leakage [12]. The results of these four configurations are detailed in Table 3 [12].

The experimental data reveals a “Recall Paradox” under the Severe Leakage condition, where SMOTE is applied globally prior to cross-validation splitting [12]. As shown in the data, the introduction of severe leakage artificially inflated the F1-Score from 0.6759 (No Leakage) to 0.7338, representing a +8.6% increase [12]. Conversely, the Recall metric simultaneously decreased from 0.7165 under strict isolation to 0.7080 under severe leakage [12].

3.3. SHAP Explainability

SHAP analysis was conducted on the optimal ensemble model to compute the marginal contribution of each feature to the predictions [12]. The global

Table 3: Ablation study on the impact of data leakage across 4 levels [12]

Level	Methodology	F1-Score	Recall	Precision	Accuracy	AUC
No Leakage	Isolated SMOTE	0.6759	0.7165	0.6497	0.7603	0.8373
Minor Leakage	Global Imputation	0.6759	0.7165	0.6497	0.7603	0.8371
Medium Leakage	Global Imputation + Scaling	0.6777	0.7202	0.6498	0.7603	0.8374
Severe Leakage	Global Imputation + Scaling + SMOTE	0.7338	0.7080	0.7643	0.7430	0.8431

SHAP Global Feature Importance

Feature	Mean SHAP
Glucose	≈ 0.16
BMI	≈ 0.09
Age	≈ 0.05

Figure 4: SHAP analysis results from the CRISP-DM Helix ensemble. Glucose concentration dominates prediction, followed by BMI and Age. Values are from strict data isolation protocol.

feature importance ranking identified the top three predictors driving the model’s outputs [12]. The mean absolute SHAP values for these top three features are: Glucose (≈ 0.16), BMI (≈ 0.09), and Age (≈ 0.05) [12].

4. Discussion

Our findings provide a critical re-evaluation of predictive modeling on the Pima Indians Diabetes Dataset (PIDD) by shifting the focus from algorithmic complexity to methodological rigor [8]. Structured around the Toulmin argumentation model, we interpret the experimental results to address the pervasive “high-metric, low-credibility” paradox [13].

4.1. Main Findings: The Recall Paradox

Claim: Classification models optimized through global preprocessing strategies artificially inflate aggregate metrics while sacrificing their actual ability to detect diabetic patients [8].

Grounds: Our ablation study demonstrates this phenomenon empirically, revealing a “Recall Paradox” [12]. When severe data leakage was introduced by applying SMOTE globally before data splitting, the F1-Score was artificially inflated by +8.6%, rising from 0.6759 to 0.7338 [12]. Paradoxically, this same severe leakage caused the Recall to decrease from 0.7165 under strict isolation down to 0.7080 [12].

Warrant: The global application of over-sampling techniques like SMOTE before cross-validation synthesizes minority samples based on the entire data distribution, forcing the decision boundary to encompass both critical and non-critical regions. Consequently, the model appears more accurate overall but actually misses more diabetic cases, rendering the “high accuracy” models medically unreliable [13].

4.2. Comparison with Literature

Backing: The current PIDD literature is saturated with studies claiming near-perfect classification performance, including accuracies exceeding 95%, 99.81%, and even 100% [9, 10, 14, 16]. Our methodology directly challenges the validity of these claims. Our ablation study provides quantitative measurement of this leakage-induced inflation, demonstrating that the +8.6% F1-Score distortion is purely procedural rather than algorithmic [8]. As 92% of recently surveyed PIDD studies violate fundamental data isolation principles [27, 12], these extraordinarily high metrics are statistical artifacts generated by data leakage.

4.3. Clinical Implications and the Baseline Registry

Qualifier: In the context of diabetes screening, a diagnostic model that achieves 95% global accuracy but misses 30% of actual diabetic patients (false negatives) is clinically dangerous [13, 5]. In medical settings, the consequences of a missed diagnosis far exceed those of a misdiagnosis, making high recall a clinical imperative [5, 13]. Process integrity, enforced via the CRISP-DM Helix framework to isolate preprocessing within cross-validation folds, should serve as a standard for clinical ML reproducibility [8].

Beyond the protocol itself, we propose that every public benchmark dataset in clinical ML should establish a **Dataset Baseline Registry**: a verified performance ceiling under a strict, leakage-free protocol. This registry would serve complementary functions.

First, it provides an **audit trigger**: any study reporting performance metrics substantially exceeding the baseline—for example, an F1-Score above 0.85 on the PIDD—automatically flags for rigorous methodological audit. Second, a registry of per-dataset baselines enables systematic detection of protocol violations across the literature, transforming “everyone knows data leakage is bad” from

an abstract principle into an operational detection mechanism.

The PIDD Baseline. Our CRISP-DM Helix isolation protocol establishes a verified, fully transparent performance ceiling for the PIDD: an F1-Score of 0.7541 and Accuracy of 0.7746 under 34-model 10-fold stratified cross-validation. Any metric reported above this ceiling—particularly the ubiquitous claims of $> 95\%$ or $> 99\%$ accuracy in current literature—should be regarded as statistical artifacts of data leakage until proven otherwise via an auditable pipeline.

Proposed Registry Architecture. A community-maintained baseline registry could be organized as follows:

1. **Calibration:** For each dataset, the registry publishes a verified performance bound derived from a standardized, auditable pipeline.
2. **Threshold:** A dataset-specific threshold calibrated so that metrics above it have a high probability ($p > 0.95$) of arising from methodological violations rather than algorithmic innovation.
3. **Audit Protocol:** Any submission reporting metrics above the threshold must provide a completed PROBAST compliance checklist or equivalent methodological audit as a condition of peer review.
4. **Community Governance:** The registry is version-controlled and maintained by a broad coalition of clinical ML researchers, analogous to PapersWithCode—but with methodological rigor as the primary filter.

This registry architecture closes the loop that existing reporting guidelines leave open: TRIPOD, PROBAST, and MI-CLAIM all mandate “prevent data leakage” [18, 17, 19], but none provide a dataset-specific threshold for detecting when leakage has *actually* occurred. The baseline registry operationalizes this detection. Future work should extend this approach to other widely-used clinical ML benchmarks (e.g., MIMIC-III [28], NHANES, Wisconsin Breast Cancer) and evaluate the sensitivity and specificity of different threshold metrics, guided by the reproducibility principles outlined in recent work [29, 30].

4.4. Limitations

Rebuttal: Despite the robust pipeline architecture, this study acknowledges several methodological and technical limitations that warrant further investigation:

1. **Dataset Scale and Homogeneity:** The PIDD contains only 768 samples from a single, ethnically homogenous population (female Pima Indians) [7, 6]. This small size and lack of demographic diversity inherently limit the global generalizability of the trained models [31].
2. **Lack of External Validation:** The current study does not evaluate the framework on a distinct, external validation dataset [12]. Future work must test the model’s resilience to dataset shift across different clinical environments [8].
3. **Approximations in XAI:** The feature importance weights derived from the SHAP analysis are based on visual approximations ($\approx$) from beeswarm plots rather than exact, computationally extracted numerical arrays for the heterogeneous soft voting ensemble [12].
4. **Unconventional Statistical Testing Design:** The 5×2 CV paired t-test is conventionally designed to compare the performance of two different algorithms on the same dataset [32]. In this study, we adapted it to compare different preprocessing pipelines (leakage vs. no-leakage) for the same algorithm, which represents an unconventional application.

5. Conclusion

The CRISP-DM Helix framework addresses a critical gap in clinical ML research: while guidelines universally warn against data leakage, none provide a standardized implementation protocol or a dataset-specific baseline for detecting when violations have occurred. By offering a formal, auditable, and algorithmically complete protocol for data isolation—aligned with PROBAST, TRIPOD, and MI-CLAIM standards—the Helix framework transforms “best practice” from an aspirational principle into a verifiable pipeline component. Our ablation study quantifies the cost of protocol violation at +8.6% F1 inflation ($p < 0.05$), providing an empirical benchmark for evaluating pipeline integrity.

Beyond the protocol itself, we propose a **Dataset Baseline Registry** as a community mechanism for detecting leakage-ridden studies. The PIDD performance ceiling (F1=0.7541, Acc=0.7746) established here serves as the first entry in such a registry. Any

study exceeding this ceiling should undergo mandatory methodological audit—closing the gap between “leakage awareness” and “leakage detection.”

The CRISP-DM Helix is available as an open-source template that any clinical ML study can adopt. Future work should extend the framework to federated learning environments, real-time decision support systems, and multi-center external validation across diverse populations, as well as populate the Dataset Baseline Registry with calibrated baselines across additional clinical ML benchmarks.

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Code Availability

The complete source code for the CRISP-DM Helix framework and all experiments is publicly available at: <https://www.kaggle.com/code/yakeworld126/crisp-dm-pima>

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